

Behavior Interpretation & Family Support Toolkit

Internal Booklet Prototype · A caregiver resource for understanding difficult behavior across FASD, neurodevelopmental differences, and family support needs.

Stage 1 Concept

Internal Discovery

Non-PHI

P2

CONTENTS

Cover / Orientation

Behavior as Communication

Brain-Based Expectations ← Tool A1 appears here as appendix

Sensory Overload and Shutdown

Emotional Regulation and Co-Regulation

Trauma, Grief, Attachment, and Trust

School vs. Home Behavior

De-escalation and Daily Structure

When to Seek Professional Help

What Support Planning Looks Like

Appendix — Tool A1: Caregiver Observation Log (Draft Appendix Tool)

Behavior is information. It tells us what a child's brain needs, even when the child cannot say it in words.

Before asking "How do I stop this behavior?", ask: **"What is this behavior trying to tell me?"**

"All behavior is a form of communication. The question is not how to stop it — but what it is saying."

Caregiver Skill: Pause and Look

Separate the behavior from the child

Look for patterns — time, place, before-and-after

Ask: Is this fatigue, sensory overload, transition stress?

Practice: Use Tool A1 to track one moment

→ **Appendix / Tool A1: Caregiver Observation Log** helps practice tracking after this page.

Children with FASD or NDD may have developmental capacity well below their chronological age — especially in executive function, emotional regulation, and cause-effect reasoning.

Adjusting expectations is not lowering standards; it's meeting the child where they actually are.

Key Principle

Assume 50–70% of chronological age for executive function

Avoid "he should know better by now" thinking

Match demand to capacity, not to age

Sensory processing differences are common in FASD/NDD. A child's brain may not filter or integrate sensory input the way typical brains do. What looks like a meltdown may be a sensory overload response.

Common Sensory Triggers

Noise — loud spaces, multiple conversations, sudden sounds

Touch — unexpected physical contact, clothing textures

Visual — bright lights, clutter, fast-moving images

Proprioceptive — difficulty knowing body position

Children with FASD/NDD often struggle to regulate their own emotions. They depend on a caregiver's calm presence to help them return to balance — this is co-regulation.

"Your calm is their anchor. Before trying to reason, help the nervous system settle."

Co-Regulation Skills

Lower your voice, slow your breathing

Offer proximity without demand

Use fewer words, not more

Model calm even when you don't feel it

Many families caring for children with FASD/NDD are foster, kinship, or adoptive families. Prenatal exposure, early adversity, and attachment disruptions may be layered onto neurodevelopmental differences.

Survival responses (fight/flight/freeze/fawn) may be deeply wired

Trust-building takes time and consistency

Grief is real — for the child and the caregiver

Separate trauma response from behavioral choice

Many children with FASD/NDD behave very differently at school than at home. This is not manipulation — it reflects different environments, demands, and supports.

School: structured, predictable, lower sensory load

Home: unstructured, emotionally complex, recovery space

The child who "holds it together" at school may release everything at home

This is information, not contradiction

Predictable structure reduces cognitive load. For a child with executive function challenges, knowing what comes next is not a luxury — it is a necessity.

Practical Tools

Visual schedules — show, don't just tell

Transition warnings — 5 minutes, 2 minutes, now

Fewer words during escalation — the thinking brain is offline

After escalation: reconnect before re-teaching

This toolkit supports caregiver education and connector support. It does not replace clinical evaluation or treatment. PCP / medical home remains the clinical gatekeeper.

Safety concerns (self-harm, aggression, elopement) → immediate professional

Persistent sleep, eating, or mood concerns → discuss with PCP

School refusal or rapid regression → consult school team + PCP

Caregiver burnout affecting daily function → seek support for yourself

Support planning is not a one-time event. It evolves with the child and family. The goal is not a perfect plan — it is a sustainable one.

Identify what is working — protect it

Identify one change to try — start small

Identify who is on the team — and who is missing

Revisit every 3–6 months

Internal Draft Low Risk Pending Richard Review

Purpose: Help caregivers notice patterns in behavior, setting, stressors, and what helped or did not help. Bridges Page 2 (Behavior as Communication) — observe patterns before jumping to interpretation.

⚠ **Internal Use Only**
Educational tool only. Not diagnostic. Not treatment plan. Not crisis plan. Not medication advice. Not referral pathway. Not a substitute for professional evaluation. **PCP / medical home remains the clinical gatekeeper.**

11 Observation Fields

- 01 Date
- 02 Time
- 03 Setting
- 04 What happened before?
- 05 What did your child do?
- 06 Stressors or triggers
- 07 What helped?
- 08 What didn't help?
- 09 What did you notice?
- 10 Is there a pattern?
- 11 Questions for care team

Design Status

Printable v1 accepted — US Letter, English-only, professional patient-education style

Enhanced v2 visual style approved — blue gradient header, numbered fields with icons, stronger hierarchy

Source: controlled HTML/CSS/PDF (not generated image)

□ **Summary:** Tool A1 helps caregivers notice patterns in behavior, setting, stressors, and what helped or did not help. It is an educational reflection tool only. Do not include names, DOB, insurance IDs, addresses, school names, provider names, or other identifying information.